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Correct Answer

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Practice Exam - 1

Question 85 of 117



A 66-year-old woman with morbid obesity, hypertension, type II diabetes mellitus, hyperlipidemia, and obstructive sleep apnea on continuous positive airway pressure therapy, presents with progressive exertional dyspnea, orthopnea, and lower extremity edema over the last 6 days. The patient has been admitted three times in the last twelve months for acute decompensated heart failure. Home medications include aspirin 81 mg daily, atorvastatin 40 mg daily, long-acting insulin 30 units nightly, bumetanide 4 mg twice daily, carvedilol 25 mg twice daily, and losartan 50 mg daily.

On examination, blood pressure is 150/90 mm Hg, heart rate is 90 beats per minute. Body mass index is 42 kg/m². Jugular venous pressure is 14 cm H₂O. There are decreased breath sounds at the base bilaterally.

Cardiac examination reveals regular rate and rhythm without murmurs or rubs. S₄ is present. Lower extremities have 2+ pitting edema bilaterally. Labs on admission showed sodium 133, potassium 4.5, blood urea nitrogen 38, serum creatinine 1.9 mg/dL. Echocardiography reveals left ventricular ejection fraction of 65% without significant valvular disease, consistent with findings from the prior study 3 months ago.

Which of the following interventions is most likely to decrease rehospitalization for heart failure

- A. Transition from bumetanide 4 mg twice daily to torsemide 40 mg twice daily
-  B. Transition from carvedilol 25 mg twice daily to metoprolol succinate 100 mg daily
-  C. Addition of spironolactone 25 mg daily
- D. Transition from losartan 50 mg daily to sacubitril valsartan 49-51 mg twice daily
- E. Addition of sildenafil 20 mg three times daily

Testing Point: The Board-certified AHFTC specialist should know which intervention prevent hospitalizations for heart failure with preserved ejection fraction (HFpEF).

Correct Answer: C. Addition of spironolactone 25 mg daily.

Rationale: Multiple large randomized controlled trials over the previous two decades have failed to show mortality benefit in HFpEF with a specific pharmacologic agent. Although the TOPCAT (Treatment of Preserved Cardiac Function Heart Failure With an Aldosterone Antagonist) trial did not reach its primary composite endpoint (CV death, congestive HF hospitalization, resuscitated cardiac arrest), spironolactone did reduce HF hospitalizations. Therefore, therapy with spironolactone is reasonable to consider for patients with similar characteristics as those enrolled in this trial (EF $\geq$ 45%, elevated BNP or HF admission within 1 year, serum creatinine $<$ 2.5 mg/dL, potassium $<$ 5 mEq/L).

Answer A: Also, while diuretics are the cornerstone of de-congestion management in these HFpEF patients, it is not reasonable to transition from bumetanide 4 mg twice daily to torsemide 40 mg twice daily, as the latter is a less potent dose.

Answer B: Beta-blockers have not been shown to reduce heart failure hospitalizations or survival in patients with heart failure with preserved ejection fraction. It may be reasonable to titrate carvedilol up to 50 mg twice daily to improve blood pressure control as this patient's blood pressure is elevated; however, transition to metoprolol succinate 100 mg daily is unlikely to provide additional benefit or prevent rehospitalization.

Answer D: A recent analysis combining the PARAGON-HF (Prospective Comparison of ARNI With ARB Global Outcomes in HF With Preserved Ejection Fraction) and PARADIGM-HF (Prospective Comparison of ARNI With ACEI to Determine Impact on Global Mortality and Morbidity in Heart Failure) trials suggested a potential benefit of sacubitril-valsartan in patients with HF and LVEF $>$ 40% (and possibly as high as 57%), although statistical significance for the primary endpoint was not achieved. Thus, sacubitril-valsartan is not currently considered standard therapy for such patients. Answer E: Phosphodiesterase-5 inhibitors (e.g., sildenafil) have not shown benefit in HFpEF, as shown in the RELAX trial.

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